

**Laboratory Investigation Report**

Patient Name	: Mohit	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 27 Y 0 M 0 D /M	OP/IP No	: OP/101317
Max ID/Mobile	: 452286/8752005200	Collection Date/Time	: 10/Sep/2021 02:14PM
Lab ID	: 1266092100235	Receiving Date	: 11/Sep/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 11/Sep/2021
Passport No.	:		

Blood Bank**Blood Grouping and RH Factor*, EDTA**

Date	10/Sep/2021 02:14PM	Unit	Bio Ref Interval
Blood Group	A POSITIVE		
Haemagglutination			

Kindly correlate with clinical findings

***** End Of Report *****
Dr. Sangeeta Pathak, DIHBT
Head-Transfusion Med

SIN No: B2B899472, Test Performed at : 1103 - Max Hospital Saket (East Block), 1, 2, Press Enclave Marg, Saket Institutional Area, Saket, New Delhi
Max Lab - A Division of Max Healthcare Institute Ltd.

Booked at: Max Super Specialty Hospital, Saket (West Block), 2nd Floor, Powerstation by Oyo Pioneer Square, Sector 62, Gurugram, Haryana 122002, Press Enclave Road, Saket, New Delhi - 110 017, Phone: +91-11-6611 5050.

8752005200
CIN No.: L72200MH2001PLC322854
The report can be verified by scanning the Q R Code on top of the page
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Conditions of Reporting: 1. The tests are carried out in the lab with the presumption that the specimen belongs to the patient name as identified in the bill/test request form. 2. The test results relate specifically to the sample received in the lab and are presumed to have been generated and transported per specific instructions given by the physicians/laboratory. 3. The reported results are for the information and interpretation by the referring doctor only. 4. Some tests are referred to other laboratories to provide a wider test menu to the customer. 5. Max Healthcare shall in no event be liable for accidental damages loss, or destruction of specimen which is not attributable to any direct and mala fide act or omission of Max Healthcare or its employees. Liability of Max Healthcare for deficiency of services, or other errors and omissions shall be limited to fee paid by the patient for the relevant laboratory services.



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Serology

Viral Markers (Rapid)

Test Name	Result	Unit	Bio Ref Interval
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Rapid Card Test - HCV*, Serum

Immunochromatography

HCV Card Test Negative

Comment Interpretation

It is only a screening test. All reactive samples should be confirmed by HCV quantitative PCR
A non reactive result does not exclude the possibility of exposure to or infection with HCV.
Patients with auto immune liver disease may show falsely reactive results

Rapid Card Test - HIV I & II*, Serum

Immunochromatography

Rapid Card HIV I & II Negative

Comment This is only a screening test. All samples detected reactive must be confirmed by using HIV RNA PCR / three different methods.
A non- reactive test result does not exclude the possibility of exposure to an infection with HIV (Window Period) or low viral load.

Advice:

1. HIV by 3 different methods
2. Confirmatory HIV RNA Quantitative PCR.



SIN No: B2B899472, Test Performed at : 896 - Motherland Hospital, Motherland hospital Sec-119

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Serology

Viral Markers (Rapid)

Test Name	Result	Unit	Bio Ref Interval
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VDRL/RPR*, Serum

RPR(Syphilis) Slide Flocculation	Non Reactive
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Comment

Interpretation

1. It is a screening test for syphilis which is useful for following the progression of disease and response to therapy. Rising titers are of immense value in confirming the diagnosis.
2. Biological false positive reactions exhibit low titers and are seen in conditions like Viral fevers, Mycoplasma infection, Chlamydia infection, Malaria, Immunizations, Pregnancy, Autoimmune disorders & past history of Treponemal infection.
3. It is advisable to confirm the diagnosis by tests such as TPHA & FTA-ABS.
4. A definitive clinical diagnosis should not be made by result of a single test only, but should be made by taking clinical history and other laboratory findings in to account.



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Rapid Card Test - Hepatitis B Surface Antigen*, Serum

Immunochromatography

Rapid Card HBsAg Negative

Comment

Interpretation

This is only a screening test.

All reactive samples should be confirmed by 'HBsAg confirmatory test or HBV DNA PCR'.

A non – reactive does not exclude the possibility of exposure to infection with HBV (window period)

False positive results can be obtained due to the presence of other antigens or elevated levels of RF factor.

Advise: Confirmatory test 'HBsAg Confirmatory Quantitative' test followed by 'HBV DNA quantitative PCR'

Kindly correlate with clinical findings

*** End Of Report ***


Dr. Mrinalini Priyadarshini
 DCP, DNB (Path.)
 Consultant Pathologist



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